

Suicide Risk Assessment and Management

Policy Number	POL-PC-060	Owner / Department	Behavioral Health / Nursing
Effective Date	2023-07-15	Version	2.1
Last Revised	2026-02-12	Next Review	2028-02-12
Approved By	Patient Safety & Quality Committee	Applies To	All patients 12 years and older treated for behavioral-health conditions, and any patient who screens positive for suicide risk, in all inpatient units and the Emergency Department

1. Purpose

To identify patients at risk for suicide and to protect them through standardized screening, environmental safety, and level-based precautions, consistent with The Joint Commission National Patient Safety Goal NPSG.15.01.01 and the Zero Suicide framework.

2. Scope

Applies to all patients being evaluated or treated for behavioral-health conditions as their primary reason for care, and to any patient in any setting who screens positive for suicidal ideation. Screening is completed on admission, on transfer, on any change in condition, and before discharge.

3. Definitions

- **Suicidal ideation** — thoughts of engaging in behavior intended to end one's life, ranging from a passive wish to die to active intent with a plan.
- **Ligature risk** — anything that could be used to attach a cord, rope, or other material for the purpose of hanging or strangulation.
- **1:1 observation** — continuous, uninterrupted line-of-sight monitoring by an assigned staff member who remains within arm's reach as clinically indicated.
- **Validated screening tool** — an evidence-based instrument such as the Columbia-Suicide Severity Rating Scale (C-SSRS) or the Ask Suicide-Screening Questions (ASQ).

4. Risk Levels, Required Precautions, and Observation

Assign a risk level from the validated screen and the clinician's assessment. When findings could support two levels, assign the higher level. Precautions are initiated immediately and are not reduced without a provider order documenting reassessment.

Risk level	Typical findings	Required precautions
High	Active ideation with plan and/or intent; recent attempt; command hallucinations to self-harm.	Continuous 1:1 observation within arm's reach; ligature-resistant environment; remove belongings and personal items; search for contraband; behavioral-health consult same day.
Moderate	Active ideation without specific plan or intent; significant risk factors; ambivalence.	Continuous line-of-sight observation; safe-room placement; remove hazardous items; reassess at least every shift; behavioral-health consult.
Low	Passive ideation only; no plan, intent, or recent attempt; protective factors present.	Routine observation with documented check frequency; environmental review; reassess each shift and on any change in condition.

5. Immediate Management

IMPORTANT — A patient who screens at high risk must never be left alone. Initiate continuous 1:1 observation immediately, remove ligature and sharp hazards and all personal belongings from reach, and notify the attending provider and behavioral-health on-call without delay. Do not wait for the formal consult to begin precautions.

1. Complete the validated screen and clinician assessment; assign a risk level.
2. Initiate the required precautions for that level immediately, including observation and environmental safety.
3. Notify the attending provider and request a behavioral-health consultation on the required timeline.
4. Document the screen, risk level, precautions, and notifications; hand off risk level and precautions at every care transition.

6. Severity & Reporting

Events are classified and reported under POL-RM-013. An inpatient suicide, or a suicide attempt resulting in serious harm, is a sentinel event and is always SEV-1: notify the attending provider, Charge Nurse, Nursing Supervisor, Risk Management, and the Administrator-on-Call immediately and within 1 hour, and manage per POL-RM-004 (RCA² within 45 business days). The elopement of a suicidal patient is managed as a Code Green under POL-SEC-010 in addition to the notifications here. Use of restraint or seclusion for a behavioral emergency follows POL-NUR-005. A near miss (e.g., contraband or a ligature intercepted before harm) is reported within 24 hours.

7. Documentation Requirements

- Screening tool used, score, and assigned risk level, with date and time.
- Precautions initiated, observation level, and the environmental safety review performed.
- Provider and behavioral-health notifications (who, when, by what method).
- Reassessments, any change in risk level, and the order supporting any reduction in precautions.

8. References

- The Joint Commission, NPSG.15.01.01 — Reduce the risk for suicide.
- Zero Suicide framework (Suicide Prevention Resource Center / EDC).
- The Joint Commission, environmental risk reduction for ligature and self-harm hazards.
- Related: POL-NUR-005 (restraint & seclusion), POL-SEC-010 (elopement), POL-RM-004 (sentinel events), POL-RM-013 (severity & reporting).